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Unit- 8:

Role and Relationship

Role and Relationships

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- ❑ A person's roles and relationships make up the fabric of his or her life responsibility
- ❑ These areas of psychosocial function have a significant impact on health and wellness, and conversely, on other areas of function
- ❑ **Role** is the overall or total expected and observed behavior in assignment among activity or work
- ❑ **Relationship** is effective frame work of bilateral communication with one another to receive, interpret and express ideas, feelings and needs.

8.1. Communication

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- ❑ Communication is the interchange of information b/n at least two people
- ❑ It is both a human need and a form of social interaction
- ❑ **There are two types of communications:**
 1. **Verbal communication:** using language
 2. **Non verbal communication:** eye contact, facial expression, body posture and movement, gestures, and touch

Communication...

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- ❑ Nurses need to be able to assess client's ability to communicate his or her needs to provide holistic nursing care.
- ❑ Most importantly nurses need to know how to intervene for a client who has impaired verbal communication and cannot readily make his or her needs known.

Normal communication pattern

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- Speech production is a motor activity that requires the coordination of laryngeal and respiratory structures to produce the sound patterns of speech.
- **Speech** is basically composed of the function of phonation (achieved by the larynx) and articulation (achieved by the mouth)
- **Phonation-** is the process by which humans create vocal sounds through the larynx
- **Articulation:** is the enunciation (accent) of words and sentences affected by lips, the tongue and the soft palate.

Normal function of communication

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- ❑ Communication is very essential to social interaction and enables the individual to relate to people in his or her family and community.
- ❑ It aids not only in developing the function of social interaction but also supplies a means of expression for other healthy functioning: coping behaviors, self concept, expressions of values and beliefs including oneself and a higher self.

Factors affecting Normal communication

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1. Cognition and perception

- **Cognition**- the act of knowing; occurs through interaction with the world, formal education and culturalization
- **Perception**- obtaining knowledge through senses; it helps to understand the external world effectively
- The sense of vision and hearing directly affect cognition and the ability to communicate.
- If cognition is impaired, communication lacks clarity

Factors affecting Normal communication

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2. Self- concept: The way a person feels about himself or herself in relation to the world is conveyed in the person's communication.

3. Culture Values and beliefs: Comprehension and meaning vary among cultures; words and their meanings can differ among or depend on culture, values and beliefs of the person.

- Hence, communication is inhibited as a result. E.g. language barriers result in anxiety, fear and frustration

Altered communication pattern

Potentials for altered communication

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- 1. Impaired speech apparatus:** Functional impairment of the speech apparatus of the larynx, the ability to move air, the use of the tongue and the oral pharynx and the physiologic innervations to each of these structures may alter communication.

Altered communication pattern

Potentials for altered communication

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2. **Disturbed articulation:** Incoordination or decreased movements of muscles required for articulation can be related to cortical, cerebral, or cranial nerve dysfunction or from effects of drugs such as alcohol, sedatives or other medications.
3. **Illness:** Stroke, spinal injury especially to cervical, head trauma, and coma can alter level of communication either temporarily or permanently.

Manifestation of altered communication

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- **Aphasia:** It is a condition of complete or partial loss of all language modalities including an understanding of speech, reading, speaking, writing, arithmetic and expression through pantomime (gesture).
- It depends on the location and extent of damage and can range from slight slurring of speech to total loss of communication.

Manifestation of altered communication

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- **Dysarthria:** It is a totally separate speech disorder which refers to a group of speech disorders that result from a disturbance of motor control, weakness, paralysis or incoordination of the oral musculature.
- Patients with dysarthria do not have language problem but rather difficulty in saying words and sounds precisely with appropriate stress loudness, pitch and control. The result is speech that is described as “slurred”, “heavy” or “unclear”.

Application of Nursing Process

Assessment: Subjective data

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- Functional pattern identification
- Risk factor identification- from family and significant others
- The nurse usually begins assessment by reviewing patient history of person's normal communication and roles of any factors that may predispose the person to altered communication and relationships.

Application of Nursing Process

Assessment: Objective data

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- The nurse can gain through mouth, tongue and facial muscles or with any sensory deprivation.
- Minor test like instructing the patient to protrude the tongue for motor function (is there any deviation to side), note symmetry of facial movement could be used.

Application of Nursing Process

Nursing Diagnosis

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- ***Impaired verbal communication***
- ***Defining Characteristics:*** Patient is unable to speak dominant language; or verbalizes with difficulty.
- ***Related factors:*** Decreased in circulation to brain, physical barrier (tracheotomy, intubations) anatomic defect (cleft palate), psychological barriers (psychosis, lack of stimuli) cultural difference developmental or age related.

Application of Nursing Process

Patient Goals

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- The patient will communicate basic needs
- The patient will demonstrate improved ability to express self
- Client will establish an effective means of communication

Application of Nursing Process

Implementation

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- Use visual cues along with speech to enhance understanding
- Listen patiently to content of responses
- Acknowledge the client's frustration
- Initiate health teaching and referrals

2. Families and Their Relationships

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❖ ***NORMAL FAMILY FUNCTION***

- Family is a social group whose members share common values, occupy specific positions, and interact with each other over time.

❖ ***Characteristics of normal family***

- Family structure
- Family function

Family Structure

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- **Nuclear family:** Members include adult man and woman, who are married, and their children.
 - Members live under the same roof.
- **Extended family:** Members include the nuclear family as well as grandparents, aunts, uncles, and cousins.
 - May or may not live under the same roof and often consists of three or more generations.

Family Structure...

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- **Single parent family:** Composed of one parent and one or more children.
- This family structure is increasing due to high separation and divorce rates.
- **Blended (step) family:** Members include children living with one birth parent and one non-birth parent, as well as the offspring of a non-birth parent.
- Each member faces challenges of forming new relationships.

Family Structure....

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- **Cohabitated family:** Includes people living together without the formal or legal bond of marriage.
- Couples include men and women living together as trial marriage or as an alternative marriage; Gay or lesbian homosexual couples. May include children.
- **Communal family:** Includes a number of members who share a common bond such as religious affiliation, ideology, economic needs, or situations such as attending college.
- **Membership may be short-term.**

Family Structure....

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- **Other families:** Other family structures are commuter marriage (one member may live away from the rest of family due to leave for work or school), single adults living alone, etc.
- Some families adopt or care for children which are not their own.

Family Function

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□ **Family functions include:**

1. Physical provision,
2. Economic provision,
3. Sexual intimacy,
4. Reproduction,
5. Education,
6. Socialization and
7. Nurturing or support.

Normal functional family pattern

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- Meeting the family's developmental tasks and guiding family members in accomplishing tasks according to their age group.

Factoring affecting normal family function

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1. Culture, values and beliefs
2. Economic status
3. Lifestyle
4. Previous life experience, and
5. Coping and stress tolerance

Application of Nursing Process

Assessment: Subjective Data

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- ***Functional pattern identification:*** Ask the client to tell you about his/her family using the following question:
 - ▣ Who are the members of your family?
 - ▣ How are the physical needs of food, shelter, health care, etc met?
 - ▣ What is the financial status of the family?
 - ▣ What are the roles of different members?
 - ▣ Who makes decisions in a family?
 - ▣ Are members support each other? etc

Application of Nursing Process

Assessment: Subjective Data

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- ***Risk identification:*** Ask for potential sources of stress in the family using the following questions:
 - Are there any financial problems in the family?
 - How healthy are other family members?
 - Is there a history of alcoholism in your family?
 - What other support system do you have?
 - Are there relationship problems among members?
 - Are there communication problems in the family?

Application of Nursing Process

Assessment: Subjective Data

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- ***Dysfunction identification:*** Ask the client to tell you about the problems in the family using the following questions
 - ▣ Are you experiencing role strain?
 - ▣ Describe the abuse you are experiencing
 - ▣ How long have you been separated?
 - ▣ What needs of your family are going unmet? etc

Application of Nursing Process

Assessment: Objective Data

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- Observe family interactions and individual members' behaviour.
- Observe behavioural signs of dysfunction like labile emotions, withdrawal, irritability, poor sleeping and eating, inability to concentrate, and dependency.
- Using physical asst techniques assess indicators of physical abuse like injuries or bruises, enlarged liver (sign of chronic alcoholism), signs of stress like weight loss, fatigue, and impaired cognitive function, etc.

Application of Nursing Process

Nursing Diagnoses

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1. *Altered family process*
2. *In effective family coping: Disabling*
3. *In effective family coping:
Compromised*
4. *Altered parenting*

Application of Nursing Process

Nursing Interventions

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- Nursing interventions to promote family health and function
 - ▣ Identifying and reinforcement of family strengths
 - ▣ Family support
 - ▣ Anticipatory guidance (support plus teaching focusing on what will happen rather than what has happened)
 - ▣ Help the family with problem solving
 - ▣ Offer referrals
 - ▣ Suggest family counselling as necessary

Loss and Grieving

Normal Greif Function

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- **Loss:** is experience of parting (separating) from an object, person, belief, or relationship that is valued.
- It is normal part of human relationship.
- May be minor as loss of wallet or major as loss of loved person.
- The intensity and duration of grief depends on factors like: Significance of the loss; Person's personality; and Availability of support.
- Each loss has both positive and negative aspects.

Types of Loss

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1. Objective versus Subjective
2. Actual versus Perceived
3. Material versus Psychological
4. Expected versus Unexpected

Types of Loss

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- **Objective loss:** loss that is readily recognized by others as a loss.
- **Subjective loss:** loss that is not readily apparent to others but perceived by the person only
- **Material loss:** loss of some tangible object or possession
- **Psychological loss:** loss of something that has no physical form but has important symbolic meaning.

Types of Loss....

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- **Objective or material loss** include loss of money, loss of home, loss of job, loss of body part, loss of body function, etc.
- **Subjective or psychological losses** include loss of relationship, loss of hope, loss of dream, loss of sense of immortality(living forever or deserving to be remembered forever), etc.

Grief

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- Grief is a psychological and physiological responses a person experience after loss of significant person, object, belief, or relationship that is valued.
- Includes physical, psychological, cognitive and behavioural responses to loss.

Grief....

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- **Bereavement:** a state of desolation (loneliness) that occur as a result of loss, particularly loss of significant person, w/c includes emotional, social, physical and cognitive responses to loss.
- **Mourning:** socially prescribed behaviour after the death of significant other like wearing black cloths.
- It varies from culture to culture.

Types of Grief

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- **Conventional grief:** psychological and physiological responses after actual loss of significant person, object, belief or relationship.
- **Anticipatory grief:** psychological and physiological responses a person shows to impending loss (real or imagined) of significant person, object, belief or relationship.
- Anticipatory grief facilitates coping when the loss actually occurred.

Models of Grief....

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1. Kubler- Ross' model: it proposes five stages of grief.

1. Stage I: denial
2. Stage II: anger
3. Stage III: bargaining (compromise and agree)
4. Stage IV: depression
5. Stage V: acceptance

Models of Grief...

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2. Grief cycle Model: proposes four phases of grief.

1. Phase I shock
2. Phases II protest
3. Phase III disorganization
4. Phase IV reorganization

Normal Functional Grief Pattern

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- Grief helps in making the following:
 - ▣ To make the outer reality of loss in to an internally accepted reality
 - ▣ To sever (cut) the emotional attachment to the lost person or object
 - ▣ To make it possible for the grieved person to become attached to other person or object
- Although grief is painful, it is both normal and necessary.
- Inability to grief is abnormal.

Factors Affecting Normal Grief Function

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- Factors that affect normal grief include:
 - Characteristics of the loss;
 - Personal resources;
 - Personal stressors;
 - Socio-cultural resources; and
 - Socio-cultural stressors.

Altered Grieving Function

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- **Dysfunctional grief:** is grief that falls outside the normal response range and may be manifested as exaggerated, prolonged, or absence of grief.
- D/t terms used to label dysfunctional grief and include pathologic grief, unresolved grief, and prolonged grief.
- **Potentials for altered grief** are several concurrent losses, few or absent support systems, poor coping skills, and previous unresolved loss.

Manifestations of Altered Grief Function

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- *Normal grief manifestations:* The following are normal at 1 year post bereavement:
 - Excessive or persistent expression of affect
 - Inability to express joy
 - Clinical symptoms of depression
 - Inability to form new relationships
 - Inability to speak of the deceased without intense emotion
 - Hearing or seeing the deceased (recently dead)
 - Feelings

Manifestations of Altered Grief Function.....

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- *Abnormal grief manifestations:* The following are abnormal *if present beyond 3 year:*
 - ▣ Leaving the deceased's room and belongings intact
 - ▣ Reporting physical symptoms similar to those the deceased had before death
 - ▣ Talking about the loss as if it just happened now
 - ▣ Inability to remember or talk about the deceased
 - ▣ Being preoccupied by the thoughts of the deceased
 - ▣ Talking or acting as if the deceased were still alive

Application of Nursing Process

Assessment: Subjective Data

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- ***Functional pattern identification:*** Assess the nature of the relationship with deceased and the circumstances of the loss.
- ***Risk identification:*** Assess and analyze the resources and stressors that the patient experiencing
- ***Dysfunction identification:*** Arrange several contacts with the patient over extended period of time and if the patient remains in complete denial or grief symptoms he/she is likely having dysfunctional grief.

Application of Nursing Process

Assessment: Objective Data

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- Objective manifestations are dejected physical appearance, slowed motor function, weeping, outburst of anger, emotional blunting, unkempt (non smart) appearance, sleep disturbance, appetite disturbance (excessive weight loss or gain) and preoccupation with thoughts of that which was lost.

Application of Nursing Process

Nursing Diagnoses

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- ***Anticipatory grieving:*** Individual or group experience of feelings in response to an expected significant loss.
- ***Dysfunctional grieving:*** Individual or group experience of prolonged or unresolved grief or engage in detrimental activities.

Application of Nursing Process

Nursing Interventions

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- Nursing interventions to health and grieving function
 - ▣ Patient teaching
 - ▣ Working through grief stages with the patient
 - ▣ Support groups
 - ▣ Care for the deceased (post mortem care)

Application of Nursing Process

Nursing Interventions

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- Nursing interventions for altered grief function
 - Individual therapy according to their feelings
 - Professionally led support group

CARE OF THE TERMINALLY ILL PATIENT AND POST MORTEM CARE

Care of the terminally ill patient

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- A patient needs intensive physical and emotional support as he approaches death.

Purpose

- To provide personal hygiene
- To provide spiritual support
- To support the family

□

Models of Grief

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1. Engel's Model: this model proposes six phases of grief.

1. Stage I: shock and disbelieve
2. Stage II: developing awareness
3. Stage III: restitution (conduct mourning activities)
4. Stage IV: resolving the loss (focus on deceased)
5. Stage V: idealization (incorporate certain characteristics of deceased)
6. Stage VI: outcome (decrease grief and interested in new relationships)

2. Kubler- Ross' model: it proposes five stages of grief.

1. Stage I: denial
2. Stage II: anger
3. Stage III: bargaining (compromise and agree)
4. Stage IV: depression
5. Stage V: acceptance

3. Parke's model: proposed four stages.

1. Stage I numbness (no feeling)
2. Stage II yearning (longing the deceased)
3. Stage III disorganization
4. Stage IV reorganization

4. Grief cycle Model: proposes four phases of grief.

1. Phase I shock
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4. Phase IV reorganization

Factors affecting normal grief function

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- ***Factors that affect normal grief include:***
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 - ▣ Personal resources;
 - ▣ Personal stressors;
 - ▣ Socio-cultural resources; and
 - ▣ Socio-cultural stressors.

Manifestations of Altered Grief Function

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Equipment

- Clean bed linens, clean gowns, gloves
- Water-filled basin
- Soap and washcloth
- Towels, lotion, linen-saver pads
- Lemon-glycerin swabs
- Petroleum jelly
- Suction and resuscitation equipment, as necessary
- Optional: indwelling urinary catheter

Procedures

1. Introduce self and verify client's identity.
2. Explain the procedure to the client
3. Gather appropriate equipment.

Meeting physical needs

4. Take vital signs often, and observe for pallor, diaphoresis, and decreased LOC
5. Reposition the patient in bed at least every 2 hours because sensation, reflexes, and mobility diminish first in the legs and gradually in the arms.
6. When the patient's vision and hearing start to fail, turn his head toward the light and speak to him from near the head of the bed

7. Change the bed linens and the patient's gown as needed
8. Observe for incontinence or anuria, the result of diminished neuromuscular control or decreased renal function.
- ✓ Provide necessary care: perineal care, bed bath, catheterize

9. With suction equipment, suction the patient's mouth and upper airway to remove secretions
 10. Offer fluids frequently, and lubricate the patient's lips and mouth with petroleum jelly or lemon-glycerin swabs to counteract dryness.
 11. If the comatose patient's eyes are open, provide eye care to prevent corneal ulceration.
 12. Provide ordered pain medication as needed
- ✓ Should be given I.V., if possible, for optimum results.

Meeting emotional needs

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- 13. Fully explain all care and treatments to the patient even if he's unconscious because he may still be able to hear
- 14. Allow the patient to express his feelings
- 15. Notify family members, if they're absent, when the patient wishes to see them

16. Offer to contact a member of the clergy or social services department, if appropriate
17. Record changes in the patient's vital signs, intake and output, and LOC. Note the times of cardiac arrest and the end of respiration, and notify the physician when these occur.

Postmortem care/Care after Death

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- ❖ Postmortem care involves the physical caring for the body after death, respecting as much as possible the wishes of the deceased and family

Purpose

- ✓ To show respect for the dead
- ✓ To prepare the body for morgue
- ✓ To prevent spread of infection
- ✓ To show kindness to the family
- ✓ To preserve the natural appearance of the body for the family and relatives

Equipments

- Gauze or soft string ties, gloves
- Chin straps, ABD pads, cotton balls, plastic shroud or body wrap
- Three identification tags
- Adhesive bandages to cover wounds or punctures
- Plastic bag for patient's belongings
- Water-filled basin, soap, towels, washcloths, stretcher

Procedure

1. Note the exact time of death and chart it
2. If the doctor is present call him to pronounce death
3. Document any auxiliary equipment, such as a mechanical ventilator, still present. Put on gloves.
4. Place the body in the supine position, arms at sides and head on a pillow. Then elevate the head of the bed 30 degrees to prevent discoloration from blood settling in the face.

5. If the patient wore dentures and your facility's policy permits, gently insert them; then close the mouth.
 - ✓ Close the eyes by gently pressing on the lids with your fingertips
 - ✓ If they don't stay closed, place moist cotton balls on the eyelids for a few minutes, and then try again to close them.
 - ✓ Place a folded towel under the chin to keep the jaw closed.

6. Remove all indwelling urinary catheters, tubes, and tape, and apply adhesive bandages to puncture sites. Replace soiled dressings.
7. Collect all the patient's valuables to prevent loss. If you're unable to remove a ring, cover it with gauze, tape it in place, and tie the gauze to the wrist to prevent slippage and subsequent loss.

8. Clean the body thoroughly, using soap, a basin, and washcloths. Place one or more ABD pads between the buttocks to absorb rectal discharge or drainage.
9. Cover the body up to the chin with a clean sheet.
10. Offer comfort and emotional support to the family and intimate friends. Ask if they wish to see the patient. If they do, allow them to do so in privacy. Ask if they would prefer to leave the patient's jewelry on the body.

11. After the family leaves, remove the towel from under the chin of the deceased patient. Pad the chin, and wrap chin straps under the chin and tie them loosely on top of the head. Then pad the wrists and ankles to prevent bruises, and tie them together with gauze or soft string ties

12. Fill out the three identification tags. Each tag should include the deceased patient's name, room and bed numbers, date and time of death, and physician's name. Tie one tag to the deceased patient's hand or foot, but don't remove his identification bracelet to ensure correct identification

13. Place the shroud or body wrap on the morgue stretcher and, after obtaining assistance, transfer the body to the stretcher.
- ▣ Wrap the body, and tie the shroud or wrap with the string provided. Then attach another identification tag to the front of the shroud or wrap, and cover the shroud or wrap with a clean sheet.
 - ▣ If a shroud or wrap isn't available, dress the deceased patient in a clean gown and cover the body with a sheet.

14. Place the deceased patient's personal belongings, including valuables, in a bag and attach the third identification tag to it.
15. If the patient died of an infectious disease, label the body according to your facility's policy.

16. Close the doors of adjoining rooms if possible. Then take the body to the morgue
17. Although the extent of documentation varies among facilities, always record the disposition of the patient's possessions, especially jewelry and money. Also note the date and time the patient was transported to the morgue.

